

Culturally Competent Patient Care

Delivering Holistic Patient Care: Working with Interpreters • E. Vanessa Spearman-McCarthy, MD • 30-slide current deck

HPC supplemental

4 official objectives

LEP communication

Professional interpreters

INTERPRET framework

SOURCE PRIORITY: the current 30-slide Spearman-McCarthy deck controls the actual lecture content and practical recommendations. The matching AI handout is secondary. Its main added value is that it explicitly expands the INTERPRET mnemonic named in the learning objective; those mnemonic labels are marked as Handout-organized framework rather than silently presented as slide headings.

Official Learning Objectives

1. Recognize barriers faced by patients with limited English proficiency and strategies to improve communication.
2. Demonstrate effective, empathetic verbal and non-verbal communication through an interpreter while respecting privacy, dignity, and culture.
3. Understand the health system's legal obligation to patients with limited English proficiency and strategies for outpatient and inpatient care.
4. Distinguish interpretation from translation and practice working effectively with an interpreter through the INTERPRET Framework.

WHAT THIS LECTURE IS REALLY TESTING

This is less about memorizing demographic statistics and more about whether you can **run a safe, respectful interpreted encounter**: choose the right interpreter, speak to the patient rather than the interpreter, pace communication appropriately, use teach-back, and document language-access decisions.

Slides 4-9: Why Language Access Matters

In the United States. . .

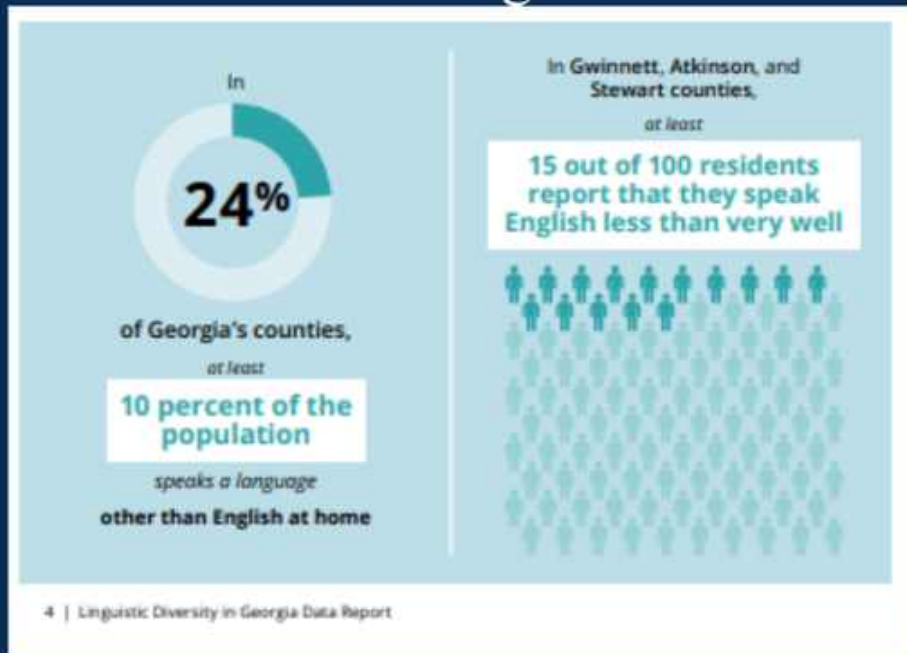
- Roughly 67.8 million (1 in 5) United States residents speak a language other than English at home
- Spanish is generally the number-one language request at most hospitals in the United States

Language	Number of speakers
Spanish	43,369,734
Chinese (incl. Mandarin, Cantonese)	3,531,221
Tagalog (incl. Filipino)	1,803,005
Vietnamese	1,570,595
Arabic	1,423,225
French (incl. Cajun)	1,194,555
Korean	1,079,420
Portuguese	1,091,374
Russian	998,179
Haitian	973,378

Languages spoken in the United States 2023 | Statista

Slide 4. The lecture opens by emphasizing that language diversity is common in U.S. healthcare: roughly 67.8 million U.S. residents speak a language other than English at home, and Spanish is commonly the most requested language in hospitals.

In Georgia



<https://www.advancingjustice-atlanta.org/linguistic-diversity>

Slide 6. Georgia-specific context: language diversity is not confined to a few urban areas; the lecture uses Georgia data to show that language-access needs exist across the state.

Limited English Proficiency

- An individual where English is not their primary language and has a limited ability to speak, read, write, or understand the English language.

<https://www.cms.gov/Outreach-and-Education/MLN/WBT/MLN2059239-Language-Access-Plans/lap/lesson01/04-Limited-English-Proficiency-LEP/index.html#:~:text=According%20to%20the%20U.S.%20Census,of%20access%20and%20communication%20barriers.>



Slide 9. Definition used in the current deck: limited English proficiency refers to a person whose primary language is not English and who has limited ability to speak, read, write, or understand English.

NEED TO RECOGNIZE

LEP is not the same thing as “speaks no English.” A patient may appear conversational in English and still be unable to safely understand a complex history, consent discussion, medication change, or discharge plan.

Clinical trap: slide 15 specifically warns that patients who speak some English may be reluctant to admit they do not fully understand. Do not use apparent conversational fluency as proof that professional language assistance is unnecessary.

Slides 10-11 and 17-18: Cultural & Linguistic Responsibility

CULTURALLY AND LINGUISTICALLY APPROPRIATE SERVICES (CLAS)

What is CLAS?

CLAS is a way to improve the quality of services provided to all individuals, which could ultimately help reduce health disparities. CLAS is about respect and responsiveness: Respect the whole individual and Respond to the individual's health needs and preferences.

Health disparities in our nation are well documented. Providing CLAS is one strategy to help eliminate health disparities. By tailoring services to an individual's culture and language needs, health professionals can help bring about positive health outcomes.

The provision of health services that are respectful of and responsive to the health beliefs, practices, and needs of patients can help close the gap in health outcomes.

<https://thinkculturalhealth.hhs.gov/clas/what-is-clas>

 AUGUSTA UNIVERSITY

Slide 11. The lecture introduces Culturally and Linguistically Appropriate Services (CLAS) as an approach centered on respect and responsiveness to an individual's health needs and preferences.

CLAS PRINCIPLE

Respect the whole person and respond to the individual's health needs and preferences. Language access is part of quality and equity, not an optional courtesy.



Slide 18. The 'Why Interpreters?' slide ties clear communication to diagnosis, treatment, compliance, safety, liability, legal requirements, CLAS standards, and accreditation expectations.

Handout legal anchor: the companion identifies **Title VI of the Civil Rights Act of 1964** as the core federal language-access obligation for federally funded healthcare settings. The current slide visually lists Title VI among the reasons professional interpretation is required, but it does not provide a detailed legal lecture.

Slides 12-13: Translation vs Interpretation

Translation vs Interpretation

- **Translation** is focused on written content, while interpretation is focused on spoken content.
- **Translation** requires high level of accuracy and can take time to produce
- **Interpretation** is delivered immediately and prioritizes understanding and communication over perfection.
- **Interpreting is the oral reproduction of spoken language**, while translation is more exact and is done in writing.



Slide 13. The deck distinguishes written translation from real-time spoken interpretation.

Translation	Interpretation
Written content	Spoken communication
High level of accuracy; can take time	Delivered immediately
Written reproduction can be highly exact	Prioritizes accurate understanding and communication in real time

FAST DISCRIMINATOR

Translate text. Interpret speech.

Slides 14-16: Why Use a Professional Interpreter?

Benefits of Using Professional Interpreters

- Better Health Outcomes due to a more accurate and timely diagnosis
- Better Access to Care and increased patient satisfaction
- Fewer Errors and reduced adverse events
- More cost-effective diagnosis and treatment
- Improved adherence with medication and treatment plans.



Slide 14. Benefits listed in the lecture include more accurate and timely diagnosis, better access and satisfaction, fewer errors/adverse events, more cost-effective care, and improved adherence.

Professional interpretation improves...	Why that matters clinically
Diagnostic accuracy	Symptoms/history are less likely to be distorted or omitted
Access and satisfaction	The patient can participate meaningfully rather than passively
Safety	Fewer misunderstandings around treatment, consent, medication, and follow-up
Adherence	The plan is more likely to be understood correctly

Methods of Interpretation

- Face to face
- Video Call
- Telephone Call



Slide 16. The current deck recognizes three major methods: face-to-face, video, and telephone interpretation.

Handout-only best-practice emphasis: use a trained/certified medical interpreter rather than relying on children or untrained family members. The current lecture's review vignette directly tests that using a family member just because an interpreter is delayed is not the preferred approach.

Slides 19-20: Before the Visit

Before the Visit

- Use a trained interpreter. Interpreters should be trained and certified in medical interpreting, especially when working in a clinical setting
- Treat the interpreter as a respected healthcare professional
- Allow extra time for the visit. Everything will be communicated at least twice (once by the speaker and once by the interpreter), unless using simultaneous interpretation
- Ensure that there are no (or minimal) distractions, such as noises that may interrupt your full engagement with the patient
- Give the interpreter a brief summary of the individual, goals, and/or procedures for the session
- Document the name of the interpreter and their certification number in the medical record.



Slide 20. The faculty checklist before an interpreted encounter: trained interpreter, professional respect, extra time, minimal distractions, brief pre-session summary/goals, and documentation of interpreter identity/certification.

BEFORE THE VISIT

Prepare the interpreter and prepare the environment.

- Use a **trained interpreter**, especially in a clinical setting.
- Treat the interpreter as a respected healthcare professional.
- Allow **extra time**: most content has to be communicated twice.
- Reduce distracting noise/interruption.
- Give the interpreter a brief summary of the patient and the **goals/procedures** for the encounter.
- Document the interpreter's **name and certification number** in the medical record.

Slides 21-22: During the Visit

During the Visit

- Introduce yourself and have others in the room introduce themselves directly to the patient upon entering the room, allowing the interpreter to interpret the greeting. Do not address your introductions to the interpreter. Introductions help set the tone and establish you as the one directing the interaction
- Use first person, and ask the interpreter to do the same
- Face and speak directly to the patient. Even if the patient maintains eye contact with the interpreter, you should maintain eye contact with the patient, not the interpreter
- Observe and monitor your and your patient's nonverbal communication



Slide 21. The most important relational rule: introductions and conversation are directed to the patient. Use first person, face the patient, maintain patient eye contact, and observe nonverbal communication.

PATIENT-FOCUS RULE

You are talking to the patient, not to the interpreter.

Say: "How are you feeling?"

Not: "Ask her how she is feeling."

Ozzy checkpoint - click him once speaking directly to the patient, in first person, feels automatic.

During the Visit

- Speak clearly, being careful not to raise your voice or shout
- Use simple language and avoid medical or healthcare jargon
- Use sentence-by-sentence interpretation. Multiple sentences may lead to information being left out
- Allow the interpreter to ask open-ended questions, if needed, to clarify what an individual says
- Observe what is going on before interrupting the interpreter. Interruption may be warranted, for example, if the interpreter is taking a long time to interpret a simple sentence, or if the interpreter is having a conversation with the patient outside their role



Slide 22. Communication technique during the visit: speak clearly without shouting, use simple language, avoid jargon, interpret sentence-by-sentence, permit clarification, and observe before interrupting.

Do	Avoid
Speak clearly at a normal volume	Raising your voice as if volume fixes language mismatch
Use simple, direct language	Medical jargon, idioms, long complex sentences
Pause sentence by sentence	Delivering an entire paragraph before interpretation
Let the interpreter clarify when needed	Assuming every long exchange is inappropriate
Intervene when the interaction moves outside the interpreter role	Losing control of the medical interview entirely

Slide 23: Wrap-Up & Documentation

Visit Wrap-Up and Important Tips

- Use the “teach back” method to rephrase and confirm that the patient understands your directions and recommendations
- Allow time for the patient to ask questions and seek clarifications
- Some individuals who require an interpreter may understand English well. Comments you make to others might be understood by the patient.
- If your patient declines language assistance services, ask them to sign a form that says they understand that language assistance is available and choose to decline these services
- This form must be available and signed in their primary language, or completed orally if they are unable to read in their primary language
- Document that the individual has been notified of these rights, as well as the patient's preferences in utilizing language access services are precautions in case of issues regarding whether certain information was provided to and understood by the patient



Slide 23. The lecture closes the encounter with teach-back, time for questions, awareness that patients may understand more English than they speak, and careful documentation when language assistance is declined.

TEACH-BACK

Do not ask only, **“Do you understand?”**

Ask the patient to explain the plan back in their own words through the interpreter.

- Allow time for questions and clarification.
- Be careful with side comments: the patient may understand more English than you assume.
- If language assistance is declined, the current deck says to document that assistance was offered and the patient's preferences.
- The deck also describes a decline form in the patient's primary language, or oral completion if the patient cannot read that language.

INTERPRET Framework

Important source note: the current deck names the **INTERPRET Framework** in its learning objective, but slides 19-23 teach the behaviors as *Before / During / Wrap-Up* checklists rather than spelling out each letter. The matching handout organizes those same behaviors into the mnemonic below.

I - Introduction

Introduce yourself and the interpreter.
Establish confidentiality and position the interpreter so the clinician-patient relationship remains central.

N - Note Goals

Briefly pre-session with the interpreter.
State the purpose and goals of the encounter.

T - Transparency

Everything said should be interpreted.
Avoid side conversations; keep statements short, simple, and jargon-free.

E - Ethics

Respect interpreter confidentiality and professional ethics; protect the patient's privacy and dignity.

R - Respect Beliefs

Remain attentive to cultural beliefs and values without stereotyping. Cultural context can be clarified when clinically relevant.

P - Patient Focus

Talk to the patient. Use first person, eye contact, and direct clinician-patient communication.

R - Retain Control

You remain the clinician directing the interview. Pace the encounter and clarify unexpectedly long exchanges appropriately.

E - Explain

Use teach-back to verify that instructions, risks, recommendations, and follow-up are actually understood.

T - Thank

Thank the patient and interpreter. The handout also recommends a brief post-session/debrief when useful.

INTERPRET IN ONE LINE

Introduce • Note goals • Transparency • Ethics • Respect beliefs • Patient focus • Retain control • Explain • Thank.

Slides 26-28: Faculty Review Vignette

In reviewing your outpatient schedule for the following day, you notice that your 2:00 pm patient is a 75-year-old woman who is coming in for an acute appointment due to stomach pain. She recently moved to the United States from China to help take care of her newborn grandson. She is fluent in Mandarin and has limited English proficiency. You arrange to have a certified interpreter to accompany you for her appointment. Which one of the following is the most appropriate for ensuring a meaningful experience for both the patient and you as the provider:

- a) You should face and speak directly to the interpreter.
- b) If the interpreter is late for the appointment, it is okay to use the patient's family member as the interpreter.
- c) You should speak in simple terms and speak one sentence at a time to avoid information being lost
- d) You should speak in third person when speaking to the patient.
- e) You should instruct the interpreter to only use closed-ended questions



Slide 27. The lecture's application question: a 75-year-old Mandarin-speaking woman with LEP is scheduled with a certified interpreter. The key choice tests the actual mechanics of communication.

BEST ANSWER

Speak in simple terms and one sentence at a time so information is not lost.

Answer choice idea	Why it is right/wrong
Face and speak directly to interpreter	Wrong: patient focus requires direct communication with the patient.
Use family if interpreter is late	Wrong: the encounter was arranged with a certified interpreter; do not default to ad hoc family interpretation for convenience.
Simple terms, one sentence at a time	Correct: matches the slide-22 communication strategy.
Speak to patient in third person	Wrong: use first person.
Interpreter should use only closed-ended questions	Wrong: the deck permits open-ended clarification when needed.

 **Working Effectively With Professional Interpreters**

Key Points:

1. Explain the purpose
2. Positioning
3. First person language
4. Keep phrases short
5. Eye contact

 **STANFORD**
HOSPITAL & CLINICS

https://youtu.be/Uhzcl2JD48?si=-S_sqloaH3fVft4w

Slide 25. The embedded video-summary slide reinforces five practical points: explain the purpose, position correctly, use first-person language, keep phrases short, and maintain eye contact.

LAST 45 SECONDS

Professional interpreter • patient-facing first person • short/simple phrases • normal voice • sentence-by-sentence • teach-back • document interpreter/decline decisions.

Source hierarchy: Spearman-McCarthy 30-slide current deck is primary. Official Week 8 LOs match the deck. The “Lecture 13: Culturally Competent Patient Care” handout is secondary enrichment, especially for the explicit INTERPRET mnemonic. Video content is not expanded beyond what the slide itself identifies because no transcript was supplied.